

Bruising and Bleeding

By **Michael B. Streiff**, MD, Johns Hopkins University School of Medicine
Reviewed/Revised Sept 2025

Bruising or bleeding after an injury is normal (see also [How Blood Clots](#)). However, some people have disorders that cause them to bruise or bleed too easily. Sometimes people bleed without any obvious triggering event or injury. Spontaneous bleeding may occur in almost any part of the body, but it is most common in the nose and mouth and the digestive tract.

People with [hemophilia](#) often bleed into their joints or muscles. Most often, bleeding is minor, but it can be severe enough to be life-threatening. However, even minor bleeding is dangerous if it occurs in the brain.

Several symptoms may suggest that a person has a bleeding disorder:

- Unexplained nosebleeds (epistaxis)
- Excessive or prolonged menstrual blood flow (menorrhagia)
- Prolonged bleeding after minor cuts, blood drawing, minor surgical or dental procedures, or tooth brushing or flossing
- Unexplained skin marks, including tiny dots that may appear red or purple on light skin and brown on dark skin (petechiae), patches that appear red or purple on light skin and brown on dark skin (purpura), bruises (ecchymoses), or small blood vessels that are widened and therefore visible in the skin or mucous membranes (telangiectasias)

Skin Symptoms That Suggest a Bleeding Disorder





Telangiectasias on the Hand

This photo shows visible, tiny blood vessels (telangiectasia) on the palmar side of the hand in this person with hereditary hemorrhagic telangiectasia.

DR P. MARAZZI/SCIENCE PHOTO LIBRARY



Petechiae (Mouth)

Petechiae are small red spots as seen here in the mouth.

DR P. MARAZZI/SCIENCE PHOTO LIBRARY



Ecchymoses (Bruises)

Ecchymoses are large purple bruises seen here on the leg.

DR P. MARAZZI/SCIENCE PHOTO LIBRARY

Sometimes a laboratory test done for some other reason shows the person has a susceptibility to bleeding.

Causes of Bruising and Bleeding

Three things are needed to help injured blood vessels stop bleeding: [platelets](#) (cell-like blood particles that help in [blood clotting](#)), blood clotting factors (proteins largely produced by the liver and by certain cells that line blood vessels), and blood vessel narrowing (constriction). An abnormality in any of these factors can lead to excessive bleeding or bruising:

- [Platelet disorders](#), including too few platelets ([thrombocytopenia](#)), too many platelets, and defective platelet function
- Decreased activity of blood clotting factors (for example, due to [hemophilia](#), liver disorders, [vitamin K deficiency](#), or the use of certain drugs or medications)
- Defects in blood vessels

Platelet disorders that cause bruising and bleeding first cause small dots that appear red or purple on light skin and brown on dark skin (called petechiae). Later, if the disorder becomes severe, bleeding may occur. A decrease in blood clotting factors usually causes bleeding and bruising. Defects in blood vessels usually cause red or purple spots and patches on light skin and brown or dark patches on dark skin, rather than bleeding.

Senile purpura and purpura simplex

Most commonly, easy or excessive bruising occurs because the skin and blood vessels are fragile. Women are commonly affected with [purpura simplex](#). Older adults are commonly affected with senile purpura. Bruises tend to develop on the thighs, buttocks, and upper arms. However, people have no other symptoms of excessive bleeding, and blood test results are normal. These conditions are not serious, and no treatment is needed.

Common causes

Overall, the **most common causes** of easy bleeding include:

- Severe platelet deficiency
- Use of medications that inhibit clotting ([anticoagulants](#)), including [heparin](#), [warfarin](#), and direct oral anticoagulants
- Liver disease (causing inadequate production of clotting factors)

Platelet deficiency can be due to inadequate production of platelets by the bone marrow or excessive destruction of platelets (for example, by an [enlarged spleen](#) or certain medications or infections).

People who have a tendency to form blood clots may take [heparin](#), [warfarin](#), or direct oral anticoagulants (including [dabigatran](#), [apixaban](#), [edoxaban](#), and [rivaroxaban](#)) to decrease that tendency (see [Medications and blood clots](#)). However, sometimes these medications decrease the body's clotting ability too much, and people have bleeding and/or bruising.

The liver is a major site of blood clotting factor production, and helps to regulate blood clotting, and so people with liver disease (for example, [hepatitis](#) or [cirrhosis](#)) can have a tendency to bleed easily.

Less common causes

[Hemophilia](#) is a hereditary disorder in which the body does not make enough of one of the clotting factors. People have excessive bleeding into deep tissues such as muscles, joints, and the back of the abdominal cavity, usually following minor trauma. Bleeding may occur in the brain, which can be fatal.

Certain disorders trigger the clotting system throughout the body. Instead of causing blood clots everywhere, platelets and clotting factors are quickly used up and bleeding occurs. This process, called [disseminated intravascular coagulation](#) (DIC), can be triggered by many conditions, including severe infections, severe injury, labor and delivery, and certain cancers. People with DIC are often already in a hospital. They bleed excessively from needle punctures and often have significant digestive tract bleeding.



Some Causes of Excessive Bleeding

Cause	Examples
Platelet disorders	
	Aplastic anemia Cirrhosis if the spleen is also enlarged DIC if it progresses rapidly Hemolytic-uremic syndrome HIV infection Immune thrombocytopenia Leukemia Medications that can trigger destruction of platelets (including heparin , quinidine , quinine , sulfonamides , sulfonylureas , or rifampin) Thrombotic thrombocytopenic purpura
Decreased number of platelets	
Increased number of platelets (which often causes excessive clotting but sometimes causes excessive bleeding)	Primary thrombocythemia Chronic kidney disease Medications that can cause platelets to malfunction (including aspirin or other NSAIDs) Multiple myeloma von Willebrand disease
Clotting disorders	
Acquired	Anticoagulants (medications that inhibit clotting) including heparin ,



MSD

Copyright © 2026 Merck & Co., Inc., Kenilworth, NJ, USA and its affiliates. All rights reserved.

Inadequate platelet function